

One Drug, Many Prices: A Proposal to Reform US Healthcare Pricing

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Introduction

The US healthcare pricing system is broken. The same product is sold to different payers for wildly different prices. Pharmacy benefit managers collect undisclosed sums and hide the amounts as a tightly guarded secret¹. The largest public insurer is effectively barred from using its leverage to reduce prices. Americans usually do not know the price at the time of receiving care. This opacity is a structural driver of the price dispersion. Few other developed nations operate a pricing system this opaque.²

I propose three reforms: consolidated government negotiation, Bloc-wide pricing as a condition of market access, and a central price lookup system. Each has been tried or proposed in isolation with promising results. I show how the three could be combined into a comprehensive plan (see [FAQ 1](#)). This note is an exercise in policy design. It presents an idea to invite consideration and critique. Legislative strategy and political roadmap would be next steps that are beyond the purview of this note.

Figure 1 shows the payment gap across ten common drugs, hospital services, and outpatient procedures. For example, a colonoscopy costs \$880 at the Veterans Affairs (VA), \$1,760 under Medicare, \$5,816 under a commercial insurer, and \$19,206 for uninsured patients. This is not an anomaly. It is common for US healthcare prices to swing several-fold across payers, and the uninsured, a vulnerable group, pay the most.

Two forces determine an insurer's ability to reduce prices: its size, and the availability of alternatives. A large insurer has more leverage because losing its volume hurts providers more, but if providers can walk to another insurer offering higher rates, that leverage evaporates. Leverage is maximized when the insurer is large and irreplaceable. The three reforms below

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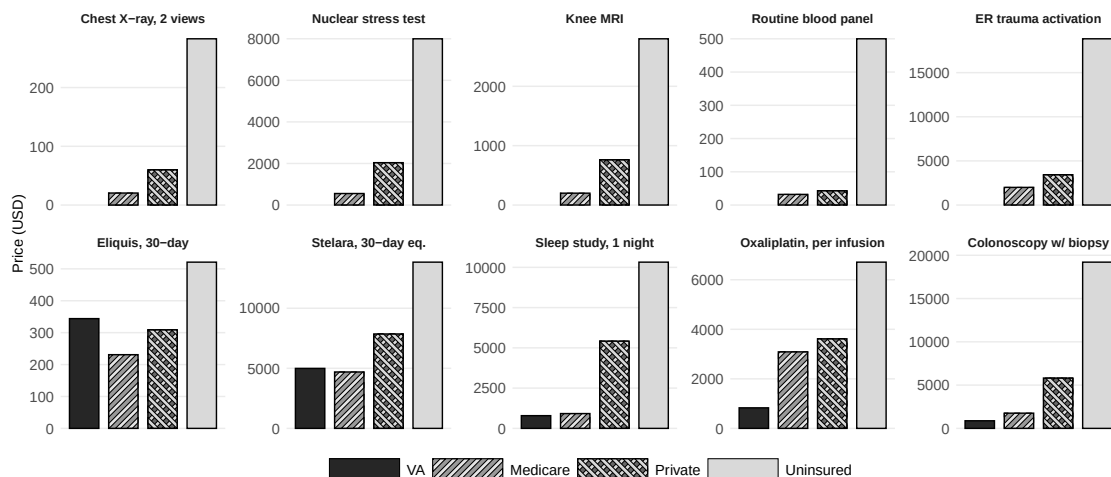


Figure 1: Price gaps across payers for the same service or product.

Notes: The "private insurer" bar is the total negotiated rate, including patient cost-sharing; the "uninsured" bar is the chargemaster or list price. Each panel uses its own vertical scale. Missing bars indicate that a reliable figure was not available in the cited sources. Full sources and derivations are on the [companion site](#).

are designed around this logic. Consolidated negotiation makes the government a single, large insurer. Bloc-wide pricing makes it difficult for providers to replace the Bloc. A central price lookup system ensures the negotiated prices are honored, not undercut by secret side deals.

Reform 1: Consolidated government negotiation

Reform 1 pools all federal healthcare purchasing into a single negotiating Bloc that bargains on behalf of every federal program. Today, public purchasing power is fragmented twice over. At the federal level, Medicare, Medicaid, and at least nine other programs each maintain their own formulary, fee schedule, and leverage. At the state level, each of the 50 states administers its own Medicaid program and its own employee health plans, adding another layer of fragmentation. For prescription drugs alone, federal and state programs combined account for roughly half of US drug spending, yet they compete against one another instead of bargaining as one.³ Hospital and physician payments are just as fragmented. Consolidating these programs into a single negotiating Bloc would use leverage the government has but wastes. Other developed nations leverage their full size in what resembles a bloc, and pay far less.^{4,5}

From Part D's launch in 2006 until 2022, Medicare was statutorily prohibited from negotiating drug prices, a restriction unparalleled in the developed world. The Inflation Reduction Act (IRA) of 2022 began to reverse it, authorizing Medicare to negotiate prices for an initial ten drugs, with more added annually. Confining negotiation to ten drugs is an arbitrary impediment that forces the government to underutilize its leverage and wastes taxpayer

money. The VA, unencumbered by that prohibition, pays half of Medicare Part D despite being much smaller.^{6,7}

Figure 2 compares prices in the US and six other developed countries for nine of those ten drugs. The US list price towers over what every peer nation pays. Even after rebates and discounts, private insurers in the US pay multiples of what patients in Canada, France, or the UK pay for the same drug. Medicare’s newly negotiated prices are 8% to 42% lower than prior net prices⁸ and narrow the gap but remain well above other nations.

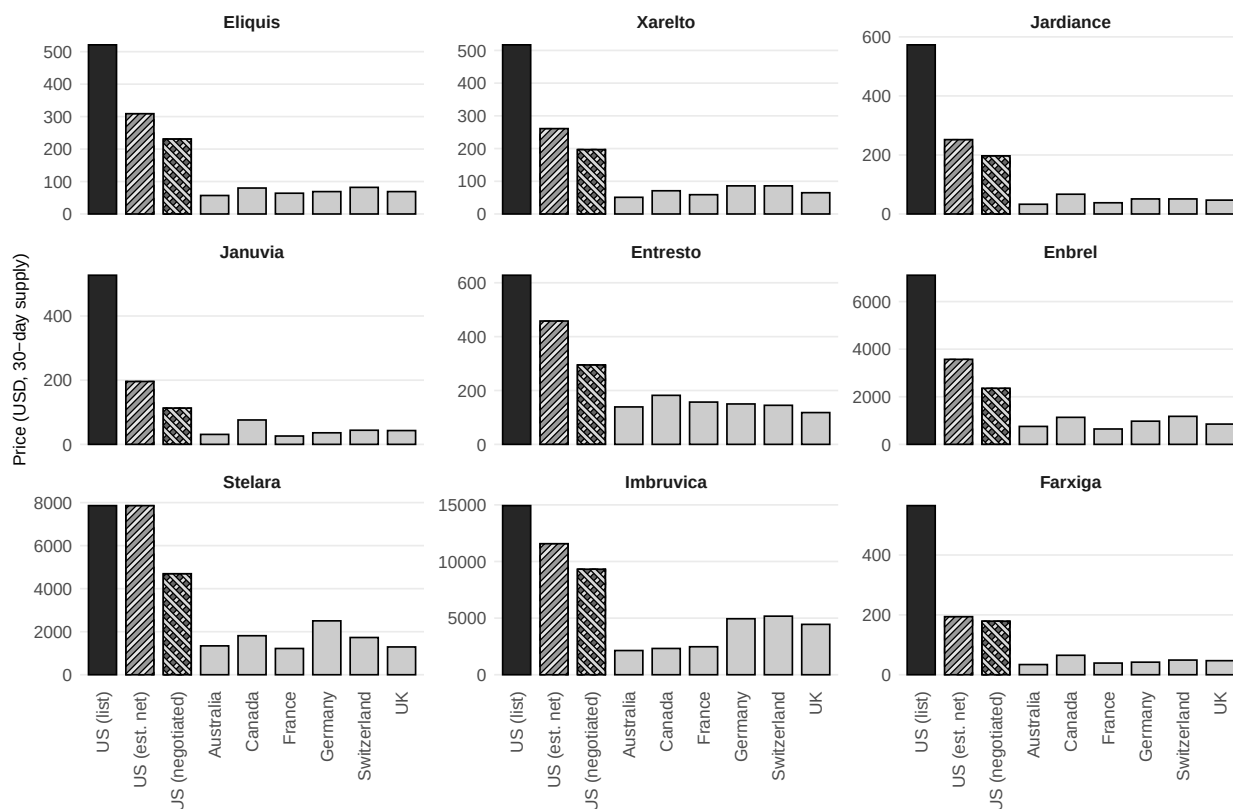


Figure 2: Drug prices by country.

Notes: The solid dark bar is the US list price (paid by the uninsured). The striped bar is the estimated US net price across all US payers combined, after rebates and discounts.⁸ The crosshatched bar is the Medicare negotiated price under the Inflation Reduction Act (effective 2026). Insulin aspart is excluded due to reporting incompatibility across countries. Each panel uses its own vertical scale. Full figures and sources are on the [companion site](#).

Reform 1 outlaws rebates, chargebacks, spread pricing, and side payments of any kind. Negotiation is conducted by salaried Bloc negotiators with expertise in pharmacology, clinical medicine, and health economics, who also make formulary and tiering decisions. Bloc negotiators are prohibited from accepting any payment beyond their government compensation package, and for twelve months after leaving the Bloc are barred from employment in any sector of the regulated healthcare industry. Compensation follows the federal Senior Executive Service (SES) model: a fixed salary at the SES level and a performance bonus capped at 10%

of base, awarded on periodic peer-reviewed evaluations rather than per-contract outcomes (see [FAQ 2](#)).

Whereas pharmacy benefit managers (PBMs) aggressively negotiate price reductions, a centralized Bloc-wide negotiation would be superior on four grounds: it eliminates the leakage that results from the secretive rebate system; instead of limiting the discount to a subset of patients and leaving others exposed to list prices, it would extend the discounted price to everyone; it eliminates the inherent conflict of interest that arises from the fact that PBMs are largely owned by health insurance companies and retail pharmacy chains;¹ and the difficulty of substituting away from the Bloc equips it with substantially more negotiation power than PBMs, which could lead to significantly lower prices. See [FAQ 7](#) for the detailed comparison.

Smaller-scale precedents for a bloc negotiation exist. The Federal Supply Schedule (FSS) publishes VA-negotiated contract prices that the Department of Defense (DoD) (including Tricare), the Indian Health Service (IHS), the Federal Bureau of Prisons, and the Public Health Service all purchase off,^{6,9} demonstrating that one body can negotiate for many federal agencies. At the state level, pooled purchasing coalitions such as the Sovereign States Drug Consortium (SSDC) extract supplemental Medicaid rebates beyond federal levels by combining state purchasing power.^{10,11}

The Bloc would be granted authority to negotiate all healthcare products and services, even those that are reasonably and comparatively priced, such as generic drugs. Universal scope does not mean universal action. Where competition produces low prices on its own, the Bloc need not negotiate. But every exemption written into the statute invites manufacturers to reformulate into the exempt category, and every exempted therapeutic class becomes the next target of pricing abuse (see [FAQ 8](#)).

Reform 2: Bloc-wide pricing as a condition of market access

Any provider that contracts with the Bloc retains the freedom to choose which private insurers to contract with, but the contracted price must be the same as the Bloc-negotiated rate. Uninsured patients cannot be refused or charged a different price. The negotiated price is locked in: no insurer can negotiate further discounts on the side. Any provider that violates these terms loses its contract with the Bloc, and with it, access to the majority of the market.

The cost problem largely lies with hospitals and pharmaceuticals, not physicians. Hospital prices for the privately insured grew 42% between 2007 and 2014, while physician prices for the same services grew only 18%.^{12–15} Yet physicians must be included in the Bloc-wide pricing. Hospitals routinely acquire physician practices and, once they do, prices rise.¹⁶

Leaving physicians outside the Bloc would create a back door for hospitals to shift billable activity to nominally independent physician groups, circumventing the Bloc’s pricing rules.

Reform 2 does not obviate insurance plans; it standardizes the price, not the coverage. Once the price of a service is fixed, insurers would compete on what fraction of that price they cover and what fraction the patient pays out of pocket. Generous plans leave the patient with a small copay, cheaper plans with more, but both pay the provider the same.

Price discrimination tends to be welfare-increasing when prices are set by buyers’ price sensitivity.¹⁷ In US healthcare, prices are set by negotiation leverage instead. If the most price-sensitive patients also have the least leverage, the standard sorting is inverted, and they end up paying the most. The uninsured face list prices,^{18,19} patients in deductible phases pay the full negotiated rate, and specialty-drug patients on coinsurance pay a percentage of list rather than net. The case against price discrimination here is therefore pragmatic, not theoretical.^{2,20} Reform 2 replaces this leverage-driven pricing with uniform pricing (see [FAQ 13](#)).

Existing attempts at constraining price discrimination have proven mostly successful. Maryland has run an all-payer hospital system since 1977.²¹ The 340B Program requires Medicaid manufacturers to sell to safety-net providers at a statutory discount (at least 23.1% off the average manufacturer price),^{22,23} though the program has left room for exploitation.²⁴ The Medicaid Best Price rule requires manufacturers to offer Medicaid the lowest price offered to any non-federal purchaser (see [FAQ 12](#) for why this is not all-payer pricing).

Reform 3: A central price lookup system

Reforms 1 and 2 produce one negotiated price per provider per service. Reform 3 makes that price visible. The Bloc operates a public price lookup system, the canonical record of every contracted price in the country. Each entry pairs a service code with a provider and a price. It updates whenever a contract is signed or revised.

Every Bloc contract pre-sets the price of every covered service, whether per service or per bundled episode. Bloc officials directly transfer prices from contract to database, with no provider input. This sidesteps the misreporting, gaming, and non-compliance that become systemic whenever transparency depends on thousands of providers posting their own data.

The lookup system is open, free, and built for comparison. A patient may compare hip replacement cost in every hospital within a chosen radius, or see EpiPen prices at every nearby pharmacy in real time. A public API lets third parties integrate prices into comparison tools,

electronic health records, and patient portals.

Physicians can use the same tool. A doctor writing a prescription for an expensive drug can identify the pharmacy with the lowest local price and send the prescription there. For costly procedures, patients may be referred across state lines if the savings justify the trip. This intensifies competition: physicians, better informed and more influential than patients in directing demand, become active routers of patients to lower-priced sellers.

Price transparency is a known driver of price reductions,^{25,26} but only if the posted prices are the actual prices and if transparency is enforced (see [FAQ 1](#)). Existing federal transparency rules have failed on both counts:²⁷ they require posting in formats too inconsistent to compare, and punish non-compliance with symbolic penalties. Under Reform 3, the Bloc posts actual prices, and overcharged patients can file a complaint. Deterrence requires large fines, for example a multiplier on the order of 100 times the overcharge (see [FAQ 3](#)), and contract termination for repeat offenders.

Final word

The reforms would benefit patients most. They would pay less, comparison-shop, and know prices before receiving care. As underlying prices fall, insured patients would face lower premiums, and uninsured patients would no longer be overcharged. The government wins too. It buys the most, so it saves the most, with a budget impact potentially in the hundreds of billions. The US has over-spent on healthcare for decades. The tools to change that are not exotic. They are familiar, tested at smaller scales, and in partial federal use. What is missing is the decision to put them together.

The [online FAQ](#) answers a set of common questions about the proposal. Comments, criticisms, and suggestions are welcome through [this link](#).

References

- [1] Federal Trade Commission. Pharmacy benefit managers: The powerful middlemen inflating drug costs and squeezing main street pharmacies. Technical report, Federal Trade Commission, 2024. URL <https://www.ftc.gov/reports/pharmacy-benefit-managers-report>.
- [2] Uwe E. Reinhardt. The many different prices paid to providers and the flawed theory of cost shifting: Is it time for a more rational all-payer system? *Health Affairs*, 30(11):2125–2133, 2011.
- [3] Centers for Medicare & Medicaid Services. National health expenditure accounts: Historical. Technical report, Centers for Medicare & Medicaid Services, 2024. URL <https://www.cms.gov/data-research/statistics-trends-and-reports/national-health-expenditure-data/historical>.
- [4] Gerard F. Anderson, Uwe E. Reinhardt, Peter S. Hussey, and Varduhi Petrosyan. It’s the prices, stupid: Why the United States is so different from other countries. *Health Affairs*, 22(3):89–105, 2003.
- [5] Michael K. Gusmano, Miriam J. Laugesen, Victor G. Rodwin, and Lawrence D. Brown. Getting the price right: How some countries control spending in a fee-for-service system. *Health Affairs*, 39(11):1867–1874, 2020.

- [6] Congressional Budget Office. A comparison of brand-name drug prices among selected federal programs. Technical Report 56978, Congressional Budget Office, February 2021. URL <https://www.cbo.gov/publication/56978>.
- [7] U.S. Government Accountability Office. Prescription drugs: Department of Veterans Affairs paid about half as much as Medicare Part D for selected drugs in 2017. Technical Report GAO-21-111, U.S. Government Accountability Office, December 2020. URL <https://www.gao.gov/products/gao-21-111>.
- [8] Olivier J. Wouters, Sean D. Sullivan, Emma M. Cousin, Nico Gabriel, Irene Papanicolas, and Inmaculada Hernandez. Drug prices negotiated by Medicare vs US net prices and prices in other countries. *JAMA*, 333(1):85–87, 2025.
- [9] Department of Veterans Affairs. VA National Acquisition Center Federal Supply Schedule pharmaceutical prices. U.S. Department of Veterans Affairs, 2026. URL <https://www.va.gov/opal/nac/fss/pharmprices.asp>.
- [10] U.S. Department of Health and Human Services, Office of Inspector General. States’ collection of offset and supplemental Medicaid rebates. Technical Report OEI-03-12-00520, HHS Office of Inspector General, December 2014. URL <https://oig.hhs.gov/oei/reports/oei-03-12-00520.asp>.
- [11] Jane Horvath. State initiatives using purchasing power to achieve drug cost containment. Technical report, National Academy for State Health Policy, April 2019. URL https://nashp.org/wp-content/uploads/2019/04/Rx-Purchasing-Paper-Jane-Horvath-FINAL-4_9_2019.pdf.
- [12] Zack Cooper, Stuart V. Craig, Martin Gaynor, Nir J. Harish, Harlan M. Krumholz, and John Van Reenen. Hospital prices grew substantially faster than physician prices for hospital-based care in 2007–14. *Health Affairs*, 38(2):184–189, 2019. doi: 10.1377/hlthaff.2018.05424.
- [13] Zack Cooper, Stuart V. Craig, Martin Gaynor, and John Van Reenen. The price ain’t right? hospital prices and health spending on the privately insured. *Quarterly Journal of Economics*, 134(1):51–107, 2019.
- [14] Gautam Gowrisankaran, Aviv Nevo, and Robert Town. Mergers when prices are negotiated: Evidence from the hospital industry. *American Economic Review*, 105(1):172–203, 2015.
- [15] Leemore Dafny. Estimation and identification of merger effects: An application to hospital mergers. *Journal of Law and Economics*, 52(3):523–550, 2009.
- [16] Zack Cooper, Stuart V. Craig, Aristotelis Epanomeritakis, Matthew Grennan, Joseph R. Martinez, Fiona Scott Morton, and Ashley T. Swanson. Are hospital acquisitions of physician practices anticompetitive? Working Paper 34039, National Bureau of Economic Research, 2025. URL <https://www.nber.org/papers/w34039>.
- [17] Hal R. Varian. Price discrimination and social welfare. *American Economic Review*, 75(4):870–875, 1985.
- [18] Stacie B. Dusetzina, Ethan Basch, and Nancy L. Keating. For uninsured cancer patients, outpatient charges can be costly, putting treatments out of reach. *Health Affairs*, 34(4):584–591, 2015. doi: 10.1377/hlthaff.2014.0801. URL <https://pmc.ncbi.nlm.nih.gov/articles/PMC4947373/>.
- [19] Ge Bai and Gerard F. Anderson. Extreme markup: The fifty US hospitals with the highest charge-to-cost ratios. *Health Affairs*, 34(6):922–928, 2015.
- [20] Gerard F. Anderson and Bradley Herring. The all-payer rate setting model for pricing medical services and drugs. *AMA Journal of Ethics*, 17(8):770–775, 2015.
- [21] Robert Murray. Setting hospital rates to control costs and boost quality: The Maryland experience. *Health Affairs*, 28(5):1395–1405, 2009.
- [22] Medicare Payment Advisory Commission. Overview of the 340b drug pricing program. Technical report, Medicare Payment Advisory Commission, May 2015. URL https://www.medpac.gov/wp-content/uploads/import_data/scrape_files/docs/default-source/reports/may-2015-report-to-the-congress-overview-of-the-340b-drug-pricing-program.pdf.
- [23] Office of the Federal Register. 42 CFR § 447.509: Medicaid drug rebates (URA). Code of Federal Regulations, 2024. URL <https://www.ecfr.gov/current/title-42/chapter-IV/subchapter-C/part-447/subpart-I/section-447.509>. Basic rebate for single-source and innovator multiple source drugs set at 23.1% of average manufacturer price.
- [24] Rena M. Conti and Peter B. Bach. The 340b drug discount program: Hospitals generate profits by expanding to reach more affluent communities. *Health Affairs*, 33(10):1786–1792, 2014. doi: 10.1377/hlthaff.2014.0540.
- [25] James C. Robinson and Timothy Brown. Increases in consumer cost sharing redirect patient volumes and reduce hospital prices for orthopedic surgery. *Health Affairs*, 32(8):1392–1397, 2013.
- [26] Christopher M. Whaley. Provider responses to online price transparency. *Journal of Health Economics*, 66:241–259, 2019.
- [27] John Xuefeng Jiang, Daniel Polsky, Jeff Littlejohn, Yuchen Wang, Hossein Zare, and Ge Bai. Factors associated with compliance to the hospital price transparency final rule: A national landscape study. *Journal of General Internal Medicine*, 37(14):3577–3584, 2022. doi: 10.1007/s11606-021-07237-y.